

HOW TO OPEN A GROUP HOME in New Jersey

A licensed community residence for adults with developmental disabilities — the N.J.A.C. 10:44A license and DDD provider approval.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New Jersey model — read this first

“Group home” in New Jersey means a community residence — a home where a small number of adults with intellectual and developmental disabilities live together and get the supervision, guidance, training, and personal support they need to live in the community rather than an institution. Two things make New Jersey distinctive, and they run through everything below. First, TWO APPROVALS: your agency must be an approved provider of the DHS Division of Developmental Disabilities (DDD), and each home must be separately LICENSED by the DHS Office of Licensing (OOL) under N.J.A.C. 10:44A, the Standards for Community Residences. Second, an EXPERIENCE GATE: New Jersey will not license you for a residence until your agency has shown at least twenty-four months of direct-care experience with the developmental-disabilities population — a requirement that is not waivable. So you almost never open a group home as your first activity.

The two ways in. Path A — start NON-RESIDENTIAL first. Deliver day habilitation, community-based supports, individual supports, or respite through the Supports Program or Community Care Program to build the twenty-four months of direct-care experience the state requires before it will license a residence. This is how most new providers begin. Path B — open the group home itself, once your agency has the experience and DDD approval in hand. For almost everyone, Path A comes first and feeds into Path B; this guide walks both and is honest about the sequence.

The names you'll deal with. DDD = DHS Division of Developmental Disabilities (approves providers, funds services through Medicaid waivers, assigns your Quality Assurance Specialist). OOL = DHS Office of Licensing (licenses and inspects each residence under N.J.A.C. 10:44A). QA Specialist = the DDD staffer who helps you build your procedure manual to standard. Support Coordinator = the person, from a Support Coordination Agency, who develops and monitors each individual's ISP and brings you placements. NJ CAT = the New Jersey Comprehensive Assessment Tool that sets a person's tier and budget. Annex A = the staff-coverage document OOL holds you to.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or licensing advice; requirements, rates, fees, and forms change, so verify each step with the Division of Revenue, DDD, the Office of Licensing, and your assigned Quality Assurance Specialist (see Part 11) before you rely on it. Where this guide gives a figure or a timeframe, treat it as a starting point to confirm with the Division, not a final answer.

The journey at a glance

These milestones take you from idea to an operating, funded New Jersey group home — with the experience runway built in.

1	Build the experience	Accumulate 24+ months of direct-care experience with the I/DD population — usually via non-residential services first
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		(Part 4)
2	Form & insure the business	Certificate of Formation with the Division of Revenue, NJ-REG, EIN, NPI, bank account, insurance (Part 3)
3	Become a DDD-approved provider	Submit the Combined Application to Become a Medicaid/DDD Approved Provider; enroll with NJ Medicaid (Part 5)
4	Secure & plan the home	Confirm zoning; plan the around-the-clock coverage you document in the Annex A (Part 6)
5	Write your policies	Build the N.J.A.C. 10:44A-2.2 procedure manual & CQI system with your QA Specialist (Part 8)
6	Programmatic audit & orientation	Pass the DDD programmatic audit; attend OOL/DDD orientation; present your business plan (Part 5)
7	Screen & train staff	Criminal-history, CARI, Central Registry, drug testing; the life-threatening-emergency curriculum (Part 7)
8	Pass the licensing inspection	Get your N.J.A.C. 10:44A license from OOL — before any individual moves in (Part 9)
9	Admit individuals & get paid	Accept Division placements, support each ISP, bill DDD/Medicaid to your Annex A (Parts 9–10)

Rule of thumb. Treat the two years not as a delay but as the apprenticeship that makes your first home succeed. Most successful residential providers spent their first stretch running day, community, or respite services — learning the Division, building relationships with Support Coordination agencies, and earning a clean compliance record. That track record is exactly what lets DDD, guardians, and families trust you with a vulnerable person's home.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is an operations-and-staffing business funded by Medicaid through the Division, not a real-estate play or a normal small business: you're responsible for a home, around-the-clock coverage, and the health, safety, and rights of the people who live there — and your revenue depends on being an approved provider, filling your licensed beds with appropriate placements the Division sends you, and staffing them to your Annex A. Second, New Jersey deliberately makes you prove experience first: the twenty-four-month direct-care requirement is not waivable, so you cannot open a group home cold as an investor. That makes it slower to enter than in-home care — but it builds a stable, contract-funded service once you're an established DDD provider.

Here's how the rest of this guide is organized: Parts 1–2 explain what a community residence is and the two approvals behind it; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 is the experience runway; Part 5 is DDD provider approval and the audit-and-orientation chain; Part 6 is the home and the Annex A; Part 7 is staffing, background checks, and training; Part 8 is the policies and records OOL inspects; Part 9 is the person's journey — eligibility, NJ CAT, the ISP, and placement — through move-in; Part 10 is how you get paid; Part 11 collects every phone number and script in one place, with FAQ and closing. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Operating a community residence — a licensed home where a small number of adults with developmental disabilities live and receive supervision, training, and personal support. Licensed by the DHS Office of Licensing under N.J.A.C. 10:44A and funded through the Division of Developmental Disabilities.*

A group home in New Jersey is a community residence: a home where a small number of adults with intellectual or developmental disabilities live together and receive the supervision, guidance, training, and personal assistance they need — help with daily living, health, medication, money, community activities, and building independence — in a home-like setting that emphasizes normalization, age-appropriateness, least restriction, and person-centered planning. By rule a community residence may house up to sixteen individuals (in practice usually far fewer), and it provides food, shelter, personal guidance, and training — it is NOT a health-care facility. It is licensed by the DHS Office of Licensing under N.J.A.C. 10:44A and funded through the Division of Developmental Disabilities. This is the model this guide is built around.

What a community residence is — and is not

- **Group home (community residence):** a provider-owned or leased home for up to sixteen individuals with developmental disabilities — in practice usually far fewer — with staff providing supervision and support; licensed under N.J.A.C. 10:44A. This is the classic group home and the focus of this guide.
- **Supervised apartment:** a more independent community-residence setting, also licensed under N.J.A.C. 10:44A, for individuals who need less intensive support.
- **Community Care Residence (CCR):** a family-caregiver model — an individual living in a caregiver's own home — licensed separately under N.J.A.C. 10:44B. Not covered by this guide.
- **Children's group home:** residential settings for children are licensed by the Department of Children and Families — a different regulator and rule set. Not covered by this guide.

Why this business — and why now

Demand is real and durable. New Jersey has thousands of adults with developmental disabilities waiting for residential placements, and the state has invested heavily in expanding community-based options over institutional ones. A well-run group home fills a genuine need — but the flip side is real regulatory structure: an experienced agency, screened and trained staff, a written procedure manual, documented staff coverage, and an Office of Licensing inspection. Meeting those standards is exactly what lets the Division, guardians, and families trust you with a vulnerable person's home and daily care. Because the demand is placement-driven rather than marketing-driven, a reliable, compliant provider that builds relationships with the Division and Support Coordination agencies can keep its licensed beds full.

Be honest about the two realities that trip up new providers. (1) *The experience gate is real: New Jersey will not license you for a residence until your agency shows twenty-four months of direct-care experience with the I/DD population — so plan to build it through non-residential services first.* (2) *This is a staffing business: around-the-clock coverage to your Annex A is your largest ongoing cost and your central safety commitment. Plan your business — and your finances — around both from the start.*

PART 2

The Two Approvals — DDD Provider Approval & the N.J.A.C. 10:44A License

In short: *Two approvals stand behind every group home: your AGENCY must be an approved provider of the Division of Developmental Disabilities, and each RESIDENCE must be separately licensed by the Office of Licensing under N.J.A.C. 10:44A. Understand how they fit together before you start.*

DDD — Division of Developmental Disabilities (your funder and approver)

DDD funds and oversees services for adults with developmental disabilities in New Jersey. To serve individuals and be paid, your agency must be an approved Medicaid/DDD provider — this establishes your standing with the Division, your Medicaid enrollment, and the programs you're approved to deliver. DDD also assigns you a Quality Assurance Specialist who helps you develop your procedure manual to standard, and it runs the programmatic audit before residential licensure. The Division — not open-market marketing — is where residential placements come from.

OOL — the DHS Office of Licensing (your licenser and inspector)

Each residence you operate must be licensed by the Office of Licensing under N.J.A.C. 10:44A, which sets the standards for the home itself, its staff, and the care delivered there. OOL inspects your residence before licensing it, and periodically and on complaint thereafter. The chapter is organized into six subchapters, and your whole operation is built to them: general provisions and licensure; organization and administration (including the procedure manual and continuous quality improvement system); advocacy and individual rights; service delivery and habilitation; health and safety, including medication and incident reporting; and fire safety and the physical environment. Your policy manual mirrors this structure, and so does the licensing inspection.

The procedure manual is required by rule. N.J.A.C. 10:44A-2.2 requires every licensee to develop and implement a written procedure manual, reviewed at least annually, designed on normalization, age-appropriateness, least restriction, and person-centered planning — with each procedure stating its title, purpose, steps, staff responsibilities, and recording requirements — plus a continuous quality improvement system. Your assigned DDD Quality Assurance Specialist helps you develop this manual to DDD standards; it is not optional paperwork but the backbone of your license.

It's worth internalizing early that the Office of Licensing and the Division are partners, not adversaries. The Division assigns you a Quality Assurance Specialist precisely because it wants your manual and your program to succeed, and the Office of Licensing inspects to a published rule rather than to surprise standards. Providers who read N.J.A.C. 10:44A closely, ask the Division questions before they guess, and build their operation to the rule from the start have a far smoother path than those who treat licensing as a hurdle to clear once and forget.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New Jersey you form the LLC through the Division of Revenue and Enterprise Services (part of the Department of the Treasury). Your entity, EIN, and NPI all have to be in place and consistent before DDD provider approval and Medicaid enrollment, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Certificate of Formation with the NJ Division of Revenue.** Online at nj.gov/treasury/revenue — the filing fee for the public-records Certificate of Formation is \$125. Name the LLC and list a registered agent with a physical New Jersey address (this can be you).
- 2. File the NJ-REG business registration.** After forming the entity, register for tax and employer purposes by filing the NJ-REG (Business Registration Application) with the Division of Revenue — this is how you get your New Jersey tax identification and register as an employer for payroll withholding and unemployment.
- 3. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your DDD provider approval, and Medicaid enrollment will reference your structure.
- 4. File your annual report.** New Jersey LLCs file an annual report with the Division of Revenue each year (a \$75 fee) to stay in good standing — calendar it so it never lapses.
- 5. Confirm local business registrations.** Check what municipal business registrations or approvals apply where you'll operate, in addition to the state filing.

No newspaper publication in New Jersey. *New Jersey has NO LLC publication requirement — forming the entity is a Division of Revenue Certificate of Formation filing (\$125), the NJ-REG registration, and the annual \$75 report. There's no separate newspaper-notice cost as some states impose.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll as a New Jersey Medicaid provider and to bill DDD / NJ Medicaid (Fee-for-Service) for the services you deliver in the home. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a developmental-disabilities community-based residential provider — confirm the exact one DDD and NJ Medicaid want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of New Jersey branches — TD Bank, Provident Bank, Valley Bank, Columbia Bank, Lakeland/Provident, PNC, or a local New Jersey credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance & bonding — what you need, who sells it, and what to ask

A group-home provider runs a 24-hour home and supports vulnerable adults, so you need coverage built for a human-services / developmental-disabilities provider — and DDD and the Office of Licensing will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage in the home, plus claims about the SUPPORT and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims against the agency	Recommended for an agency
Non-Owned & Hired Auto	Staff driving their own cars to transport the people you serve	Yes — critical
Property	Coverage on the provider-owned or leased residence and its contents	Yes — you run a home
Workers' Compensation	Direct-care staff injuries	Required in NJ (any employees)

Abuse & molestation is your highest exposure. Supporting vulnerable adults in a residential setting means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly, and confirm the limits DDD and the Office of Licensing expect.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD residential services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local New Jersey agent at trustedchoice.com and ask specifically for a developmental-disabilities community-residence provider program. Confirm the carrier writes this in New Jersey before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities community-residence providers in New Jersey — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for staff who transport the people we support?”

“What general/professional-liability and property limits do DDD and the Office of Licensing expect for a licensed residence?”

“Can you set up New Jersey workers' comp AND the required temporary disability coverage for my direct-care staff?”

New Jersey also mandates temporary disability coverage. Beyond workers' comp, New Jersey employers must provide Temporary Disability Insurance (TDI) — a state program covering non-work-related illness or injury — funded through payroll. When you set up payroll and workers' comp for your direct-care staff, set up NJ TDI (and Family Leave Insurance) at the same time so you're compliant from your first hire. Ask your payroll provider or insurance agent to configure all three together.

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves New Jersey providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your Combined Application to become a Medicaid/DDD provider, your New Jersey Medicaid enrollment, and your N.J.A.C. 10:44A license file. Your information passes through the Division of Revenue, DDD, Medicaid, and the Office of Licensing before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled approval or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your registration, provider approval, Medicaid enrollment, and license calendared for renewal — letting any one lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply for provider approval:

- Your formed New Jersey LLC (Certificate of Formation filed, NJ-REG registered, annual report current) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, property, and workers' comp (plus NJ TDI)
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you're ready to build the provider-approval application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more New Jersey applications than any substantive problem does.

PART 4

The Twenty-Four-Month Experience Requirement

In short: *New Jersey will not license you for a residence until your agency shows at least twenty-four months of direct-care experience with the I/DD population. This is not waivable — so you almost never open a group home as your first activity. Most providers build the experience through non-residential services first.*

This is the single requirement that most often surprises new entrants, so plan around it from the start. New Jersey requires an agency to show at least twenty-four months of direct-care experience serving individuals with developmental disabilities before it can be licensed to operate a residence. This requirement is not waivable, and there's no shortcut around it.

The practical consequence is that you almost never open a group home as your first activity. Instead, most new providers begin by delivering non-residential services — day habilitation, community-based supports, individual supports, or respite — through the Community Care or Supports Programs. Doing so builds the experience the state requires, establishes your reputation and relationships with the Division and Support Coordination agencies, and creates a revenue stream that can later support the residential side. Treat the two years not as a delay but as the apprenticeship that makes your first home succeed.

4.1 How to build the experience deliberately

- **Start non-residential.** Offer day habilitation, community-based, individual-support, or respite services to accumulate documented direct-care experience with adults who have developmental disabilities.
- **Document everything.** Keep clear records of the services you provide and the individuals you serve — this is the EVIDENCE of your experience that DDD looks at, so treat your service records as the proof they are.
- **Build relationships.** Work closely with Support Coordination agencies and the Division; residential placements come through the Division, not open-market marketing, so the relationships you build now are your future census.
- **Learn the system on the non-residential side.** Incident reporting, guardian notification, person-centered planning, and compliance habits you build delivering day or respite services are exactly the habits your first home will need — so build them correctly now.

The experience runway is also your business runway. The two years of non-residential services aren't just a box to check — they're a working business that generates revenue, builds your team, and earns the compliance record the Division will weigh when you apply to add a residence. Budget the experience runway (see Part 10) as the largest early investment of time and money, and use it to arrive at residential licensure already knowing how to run a compliant DDD program.

PART 5

Becoming a DDD-Approved Provider

In short: *To serve individuals and be funded, your agency must be an approved Medicaid/DDD provider. You complete the Combined Application, enroll as a NJ Medicaid provider, pass a programmatic audit, and attend orientation with the Office of Licensing and DDD — presenting your business plan for the residential program.*

Give the provider-approval process the same seriousness as the license itself — it establishes your agency's standing with the Division, your Medicaid enrollment, and the programs you're approved to deliver. Gaps or inconsistencies here slow everything downstream, so make sure your entity, ownership, and service information are complete and consistent (Part 3) before you submit.

- 1. Complete the Combined Application.** Complete the Combined Application to Become a Medicaid/DDD Approved Provider and the related attestation for the services you'll provide. This is your agency's formal request to join the DDD provider network.
- 2. Enroll as a New Jersey Medicaid provider.** Enroll with New Jersey Medicaid so the DDD services you deliver can be funded. Your EIN and Type 2 NPI must match your entity exactly.
- 3. Pass the programmatic audit.** Before residential licensure, expect a programmatic audit confirming your agency meets DDD quality and compliance expectations — this is where your documented experience and clean record pay off.
- 4. Attend orientation and present your business plan.** Attend orientation with the Office of Licensing and DDD, and present your business plan for the residential program — who you'll serve, how you'll staff and supervise the home, how you'll keep it financially stable, and how you'll meet the rule.

Your business plan is where you show the Division you understand the operation you're proposing. A credible plan reflects the real economics of around-the-clock staffing against DDD funding, not optimistic assumptions, and it's the document that turns your experience and your provider approval into a concrete, fundable home. Providers who get through cleanly treat provider approval as a quality gate, not a form — the Division wants evidence that you can safely support people, and building your policies and staffing to that standard does double duty: it earns your approval AND it's how you'll actually run.

PART 6

The Home, the Annex A & Fire Safety

In short: *The residence must be a safe, home-like place meeting the physical-environment and fire-safety standards of N.J.A.C. 10:44A. You confirm zoning early and plan around-the-clock coverage in the Annex A — the staff-coverage document the Office of Licensing holds you to.*

6.1 The residence itself

The residence must be a safe, home-like place that meets the physical-environment and fire-safety standards of N.J.A.C. 10:44A — working smoke and carbon-monoxide detection, a fire-alarm system and extinguishers, clear exits, safe utilities, adequate space and privacy, and the ability to evacuate the home promptly. Because you serve people who may need help to evacuate, fire safety and staffing are planned together, not separately.

6.2 Location and zoning

Location matters in two practical ways. First, confirm local zoning early; community residences have legal protections under New Jersey law, but you still need to clear zoning and any local approvals for the property before you commit. Second, plan the staff coverage the home will need — because a home that can't be staffed to its residents' needs can't be licensed. Confirm both the property AND the coverage before you sign.

6.3 “Do I need fire sprinklers?” — the honest answer

Fire and life-safety requirements for a community residence come from N.J.A.C. 10:44A, the New Jersey Uniform Fire Code, and your local fire official, and they depend on the building's size, occupancy classification, and layout — so a specific answer for a specific home has to come from those authorities, not a general guide. What is fixed is the safety outcome the rule demands: working smoke and carbon-monoxide detection, a fire-alarm system, extinguishers, clear and unobstructed exits, and the ability to evacuate the home promptly — with a three-minute evacuation benchmark. Because evacuation is a benchmark, staffing and fire safety are planned together: you must have enough staff on every shift to get everyone out in time. Confirm the specific fire, sprinkler, and life-safety requirements for YOUR building with the local fire official and the Office of Licensing BEFORE you buy or renovate.

6.4 The Annex A — your staff-coverage commitment

The coverage your home provides is documented in the Annex A — the staff-coverage document the Office of Licensing reviews at the licensing inspection and at contract renewal. Staffing the home to the Annex A, around the clock, is one of the most important safety commitments you make.

Staff coverage is a safety commitment, not paperwork. *The Annex A is not a document you file and forget. It states the coverage each home provides, and the Office of Licensing holds you to it. Any reduction in coverage for an individual assessed as able to be unsupervised for a set time must be supported by the interdisciplinary team's determination AND the Division's documented approval — never done quietly to save money. Quietly running below your Annex A is where preventable harm and serious findings begin.*

6.5 The home checklist

- ☐ N.J.A.C. 10:44A physical-environment and fire-safety compliance: smoke and CO detection, fire-alarm system, extinguishers, clear exits, prompt evacuation (three-minute benchmark).
- ☐ Local zoning and any municipal approvals cleared for the property before you commit.
- ☐ Safe utilities, adequate space and privacy, and a safe, clean, sanitary home.
- ☐ A completed Annex A documenting around-the-clock coverage sufficient for the individuals you'll serve.
- ☐ Fire, sprinkler, and life-safety requirements for your specific building confirmed with the local fire official and the Office of Licensing.
- ☐ Setting size respected — a community residence is licensed for up to sixteen individuals, in practice usually far fewer; staff to who you actually serve.

PART 7

Staffing, Background Checks & Training

In short: *Your individuals are only as safe as your staff. New Jersey requires layered screening — fingerprint criminal-history, CARI, Central Registry, and drug testing — plus orientation including the Division's life-threatening-emergency curriculum, all completed BEFORE a staff member has contact with the people you serve.*

7.1 The roles

- **Agency director / administrator:** runs the agency and owns provider approval, the license, and the Annex A commitment.
- **Direct-care staff:** the people who provide the around-the-clock supervision, guidance, training, and personal support in the home — your largest team and largest cost, staffed to the Annex A.
- **Nurse oversight:** for health and medication oversight per N.J.A.C. 10:44A — often part-time or contracted at first.
- **Quality / compliance:** to keep up with N.J.A.C. 10:44A, run the CQI system, and prepare for Office of Licensing inspections.
- **Administrative / billing support:** handles Medicaid/DDD billing, records, and documentation.

7.2 Layered screening — required before any contact

Screening is layered because a single check does not catch everything. New Jersey requires a fingerprint-based criminal-history background check, a Child Abuse Record Information (CARI) check through the Department of Children and Families, screening against the Central Registry of Offenders Against Individuals with Developmental Disabilities, and drug testing under Stephen Komninos' Law. Complete these before a staff member works with individuals, and act promptly on any disqualifying result.

- **Criminal-history background check:** a fingerprint-based check. You may not employ a person convicted of a disqualifying crime — including a crime involving abuse of a person with a developmental disability — except as the Rehabilitated Convicted Offenders Act permits.
- **CARI check:** assure through the Child Abuse Record Information check that no employee is listed on the Department of Children and Families' Child Abuse Registry.
- **Central Registry:** do not employ any person placed on the Central Registry of Offenders Against Individuals with Developmental Disabilities; a person placed on it must cease all contact.
- **Drug testing (Stephen Komninos' Law):** test employees for controlled dangerous substances as required under N.J.S.A. 30:6D-9.5, with written procedures assuring compliance.

7.3 Orientation and training

Complete new-employee orientation under N.J.A.C. 10:44A-2.6 — including unusual-incident reporting, emergency procedures, an overview of developmental disabilities, and the Division's life-threatening-emergency curriculum — with a signed acknowledgment, plus disability-specific training within the first ninety days. Onboard to the standard: a competency check before a staff member supports someone alone protects the people you serve and satisfies the rule.

Two laws every staff member must know from day one. Under Danielle's Law (N.J.S.A. 30:6D-5.1 et seq.), staff must call 911 in a life-threatening emergency and report the call to the Department — with civil penalties for failing to; never delay 911 to call a supervisor first. Under Stephen Komninos' Law, any employee or volunteer who fails to report suspected abuse, neglect, or exploitation is a disorderly person and may be fined \$350 for each day the violation goes unreported. Train both from the first shift.

7.4 Recruiting and keeping staff — the real make-or-break

Direct-care staff are the heart of this business and the hardest thing to keep. Turnover is high across the whole field, and a home that can't reliably staff its shifts can't meet its Annex A, can't keep people safe, and can't keep its license — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required screening and training, and a competency check before anyone supports someone alone — this protects the people you serve and satisfies the rule.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a staff member hits a hard day, and recognition. Every person who quits costs you re-hiring, re-training, and continuity for the individuals they supported — and can put your Annex A coverage at risk.

In a residential setting the stakes on retention are higher than in most human-services work, because coverage is a licensing commitment, not just a scheduling problem. When a staff member calls out on an overnight shift, you cannot simply leave a bed uncovered — the Annex A obligates you to fill it, so you need a bench of trained, screened relief staff who can step in without a lapse in the criminal-history, CARI, Central Registry, and drug-testing checks that every person must clear before contact. Build that bench deliberately: over-hire slightly against your minimum coverage, cross-train staff across your homes if you operate more than one, and keep a small pool of per-diem staff who are fully screened and current on the life-threatening-emergency curriculum so a single call-out never forces you below your committed coverage. The providers who survive their first year treat the staffing pipeline — recruiting, screening, training, and a relief bench — as the operational heart of the business, because in New Jersey a home that can't staff its Annex A is a home that can't stay open.

PART 8

Policies, Records & Staying Compliant

In short: *Your procedure manual and documentation are what the Office of Licensing inspects and what DDD funds against. Build the N.J.A.C. 10:44A-2.2 manual and CQI system with your Quality Assurance Specialist from day one — medications, emergencies, incident reporting, and the rights of the people you support.*

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — and each individual's ISP drives what you deliver and record. This documentation is both the N.J.A.C. 10:44A requirement and the backbone of a safe, well-run home. Build the procedure manual and continuous quality improvement system required at N.J.A.C. 10:44A-2.2 with your assigned DDD Quality Assurance Specialist, and keep a simple system that holds each person's ISP, service records, medication records, and incident documentation current — because the Office of Licensing inspects it and can act on complaint.

Make the manual live in practice. *The most common documentation failure isn't a missing binder — it's a manual staff can't describe or follow. The Office of Licensing wants to see that staff know and carry out the procedures they're responsible for, that medication records are accurate, that incidents are reported within required timeframes, and that guardians are notified within the required window — two hours for injuries tied to abuse, neglect, or exploitation or any moderate or major injury. Build a real, used CQI process where incident and quality data actually drive changes, not a binder that's never opened.*

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Eligibility, NJ CAT, the ISP & Move-In

In short: You can't get paid until an individual is placed with you and supported on their ISP. The path runs from DDD eligibility, through the NJ CAT assessment that sets their tier and budget, to the ISP and a Division placement into your licensed, staffed home.

- 1. The person establishes DDD eligibility.** They have a documented developmental disability that began before age 22, meet the functional and financial criteria, and — for residential/Community Care Program services — meet the ICF/IID level of care. Eligibility is determined by the Division.
- 2. They're assessed with the NJ CAT.** The New Jersey Comprehensive Assessment Tool (NJ CAT) assesses the person's support needs across self-care, behavioral, and medical domains and helps determine their tier and individual budget — the funding that follows them to a provider.
- 3. Their ISP is developed.** A Support Coordinator, from a Support Coordination Agency, develops each person's Individualized Service Plan through person-centered planning with the individual, their guardian, and the team — and monitors it over time. The ISP sets the residential goals your home carries out.
- 4. The Division matches them to a home.** Residential placements come through the Division, not the open market. When an individual is referred to your home, admit them ONLY when your home can genuinely meet their medical, behavioral, and supervision needs within its license and Annex A staffing.
- 5. Admission and move-in.** Gather pre-admission data — diagnosis, psychological evaluation as available, and developmental and behavioral history — confirm the home can meet the individual's needs, complete the residency agreement, and the individual moves in. You're now delivering a billable, funded service.

9.1 Individual rights and the record

- **Individual rights:** honor rights to dignity, privacy, choice, personal funds and possessions, family contact, and grievances without reprisal — and apply the least restriction necessary, only as permitted.
- **The individual record:** maintain an individual file at the residence (the client record is the property of the Department) with guardianship, pre-admission data, a personal-posessions inventory, authorizations, and identification of capabilities and needs.
- **Protect and report:** report suspected abuse, neglect, or exploitation immediately to the DDD hotline at 1-800-832-9173 (press 1), report unusual incidents through the Department's system, and call 911 in an emergency.

The through-line of this part is person-centeredness made real. The individuals in your home are adults with their own preferences, relationships, and goals, and the rule expects your home to support them to live as independently and as fully in the community as they can — not simply to be safe and supervised. Inspectors and Support Coordinators notice the difference between a home that manages people and one that supports them, and so, most importantly, do the individuals who live there.

Your census depends on the Division and your reputation, not marketing. Because funding follows the individual and placements come through the Division, your revenue depends on filling your licensed beds with appropriate placements and staffing them well. Get known to the Division and to Support Coordination agencies as the provider who supports people well, communicates, handles a placement smoothly, and never runs below its Annex A — and the referrals follow. Don't build a model that assumes you fill every bed the month you're licensed; assume a gradual build as the Division learns to trust you.

PART 10

How You Get Paid, Costs & Timeline

In short: A group home is funded through the Division — the Community Care Program waiver and the Supports Program — not private pay. Individuals contribute to their room and board, which is kept SEPARATE from your service funding. Model your real costs against the funding before you commit.

10.1 How the funding works

- **The Community Care Program (CCP) waiver:** the Medicaid waiver (N.J.A.C. 10:46C) that funds residential and supportive services for eligible individuals who meet the ICF/IID level of care — the primary funding path for group-home residents.
- **The Supports Program:** funds services for eligible individuals living in their own or the family home — relevant to the non-residential services you may deliver while building experience (Part 4).
- **Fee-for-service billing:** DDD moved to a fee-for-service model — you bill DDD / NJ Medicaid for the services documented in your Annex A and ISPs, using your Type 2 NPI. Bill only for services actually provided; program-integrity review is a normal part of being a provider.
- **Contribution to care:** individuals contribute to the cost of their room and board, handled under N.J.A.C. 10:46D and Division guidance; the residency agreement identifies the individual's contribution and payments.

Room and board is separate from your service funding. The waiver and the Supports Program pay for the SERVICE — the support, supervision, and habilitation you provide — not the person's rent and food. Room and board comes from the individual's own income (typically SSI), under the contribution-to-care rules, and is identified in the residency agreement. Keep the two completely separate in your books and your agreements; mixing them is a common and serious mistake.

10.2 The money math — how a group home pencils out

Group-home economics are different from an hourly private-pay business — your revenue is the DDD/Medicaid funding that follows the individuals you serve (driven by their NJ CAT tier and budget), and your biggest cost is around-the-clock staffing. Lay it out with your own numbers:

- **Revenue:** the funded services in each individual's ISP, billed to DDD/NJ Medicaid to your Annex A coverage — so a home that's fully placed with appropriately assessed individuals and staffed efficiently is a home that pencils out.
- **Cost:** around-the-clock direct-care wages and payroll taxes are the bulk; then nurse oversight, administration, the home (purchase/lease, utilities, food), insurance, and training. Most are fixed against your licensed census.
- **The lever:** because staffing to the Annex A is largely fixed, your margin comes from keeping your licensed beds filled with appropriate placements and staffing efficiently to the coverage each home needs — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the individual's rent and food come from their own income under contribution-to-care — keep that money and those agreements entirely separate from your service billing.

10.3 What it costs to start

Costs vary widely with whether you own or lease the home, its size and condition, your staffing model, and your region of New Jersey. Rather than a single number, budget by category and confirm current figures as you plan:

- **The experience runway:** the cost of operating non-residential services for the two years you build experience — often the largest early investment of time and money.
- **The home:** purchase or lease, any modifications for safety and accessibility, furnishings, and fire and life-safety equipment.

- **Staffing:** around-the-clock direct-care coverage to the Annex A, plus supervision and administration — the largest ongoing cost.
- **Screening and training:** background checks, CARI, Central Registry, drug testing, and the required orientation and training for every hire.
- **Business and compliance:** entity registration, insurance, Medicaid/DDD provider enrollment, policy development, and any consulting help for your first residence.

10.4 Timeline and the licensing inspection

Plan for a multi-year arc: two years-plus building experience through non-residential services, then several months more for DDD provider approval, the programmatic audit, orientation, home preparation, staff screening and training, and the licensing inspection. The Office of Licensing inspects your residence against N.J.A.C. 10:44A before licensing it, and periodically and on complaint thereafter — looking at your procedure manual and CQI system, personnel files and background checks, staff coverage against the Annex A, individual records, medication practices, incident reporting, rights, and fire and physical-environment safety. Only after you pass does any individual move in.

Treat your first ninety days as the period that sets your habits. The routines you build now — how you document medication, how quickly you report incidents and notify guardians, how you run fire drills across shifts, how you keep individual records current — are exactly what future inspections examine. Build them correctly from your first individual and they simply become how the home runs.

- **Have the manual live in practice:** staff should be able to describe and follow the procedures they carry out, not just point to the binder.
- **Keep records inspection-ready:** individual files at the residence, personnel files complete with all checks and training, medication records accurate, incident and drill logs current.
- **Report and notify on time:** report unusual incidents within required timeframes and notify guardians within the required window — two hours for injuries tied to abuse, neglect, or exploitation or any moderate or major injury.

Staying licensed is about consistency. Once you're licensed, keeping the license is about doing the same things well every day: keep the procedure manual current and reviewed annually, run your CQI system so incident and quality data actually drive changes, keep every staff member's screening and training up to date, and maintain the coverage you committed to in the Annex A. When the Division amends the rule or issues new circulars, update your procedures and re-train. Growth means adding residences one at a time — each separately licensed and staffed — and growing only as fast as you can staff and supervise well.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: *The Division of Revenue, DDD, the Office of Licensing, and your Quality Assurance Specialist are the center of gravity, with your local fire official close behind. Here's the list and the scripts, plus common questions and a closing.*

11.1 Statewide contacts (same for everyone)

For...	Contact & what to ask
Forming your business	NJ Division of Revenue & Enterprise Services (nj.gov/treasury/revenue). Ask: how do I file the Certificate of Formation, the NJ-REG, and my annual report?
EIN & NPI	irs.gov (EIN) • nppes.cms.hhs.gov (Type 2 NPI). Ask NPPEs: which taxonomy fits a DD community-residence provider?
Becoming a DDD provider	NJ Division of Developmental Disabilities — provider enrollment (nj.gov/humanservices/ddd). Ask: how do I complete the Combined Application, and what services can I be approved for?
Licensing your residence	NJ DHS, Office of Licensing (nj.gov/humanservices/ooll). Ask: what does N.J.A.C. 10:44A licensing require, what is the programmatic audit, and what are current timeframes?
Your policy manual	Your assigned DDD Quality Assurance Specialist. Ask: how do we develop the procedure manual and CQI system to DDD standards?
Background checks	NJ DHS/DDD background-check process and DCF CARI Unit (nj.gov/DCF). Ask: how do I complete criminal-history, CARI, and Central Registry checks and drug testing before staff have contact?
Funding & Medicaid	NJ DDD and the Division of Medical Assistance & Health Services. Ask: how are group-home services funded through the CCP waiver, and how does contribution to care work?
Reporting abuse or an incident	DDD Abuse & Neglect Hotline 1-800-832-9173 (press 1), 24/7; unusual incidents to the DDD Office of Risk Management; 911 for emergencies.
Fire & building safety	Your local fire official and construction/zoning office, with the DHS Office of Licensing. Ask: what fire, life-safety, and zoning approvals does my community residence need?

11.2 The calls that matter most — and exactly what to ask

① DDD provider enrollment — your most important call

"I want to become an approved Medicaid/DDD provider and eventually license a community residence. How do I complete the Combined Application, and what non-residential services can I start with to build my experience?"

"How does the twenty-four-month experience requirement work, and how do I document it?"

② The Office of Licensing — the license

"What does the N.J.A.C. 10:44A licensing process require for a community residence, what is the programmatic audit, and what are the current timeframes?"

"What does the licensing inspection examine, and what's the most common reason a first residence fails it?"

③ Local fire official / zoning — the home

"I'm opening a community residence for adults with developmental disabilities. What fire, sprinkler, life-safety, and zoning approvals apply to my specific building?"

11.3 Worked examples — how the path comes together

These sketches show how the path comes together in practice. They're illustrative, not prescriptive — your own timeline, costs, and staffing will differ.

- **The day-program provider who opens a first group home:** an agency has spent two and a half years providing day habilitation and community-based supports through the Supports and Community Care Programs, with real relationships with several Support Coordination agencies and a clean compliance record —

experience well past the twenty-four-month requirement. Deciding to add residential services, the owner completes the residential side of DDD provider approval, leases a four-bedroom home and confirms zoning, develops the N.J.A.C. 10:44A manual and CQI system with a DDD Quality Assurance Specialist, plans the around-the-clock Annex A coverage, screens residential staff, and completes the life-threatening-emergency curriculum. After the programmatic audit and OOL orientation, the home passes inspection and the Division places its first individuals. Because the agency built its habits during its day-program years, incident reporting, medication documentation, and fire drills are second nature from day one.

- **The experienced operator expanding carefully:** an established provider already operates two licensed group homes and adds a third to serve individuals coming off the Community Care Program waiting list. Experience and provider approval are long since in place, so the work is concentrated on the new residence — securing and preparing the home, confirming zoning and fire safety, planning and staffing the Annex A for the specific individuals expected, and adapting the existing procedure manual. The provider recruits and trains the new home's team BEFORE opening rather than after, treats the third home's first ninety days with the same care as its first, and keeps each residence separately licensed and separately accountable. Its reputation for quality is what earns it the next round of placements from the Division.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I really have to build two years of experience first?

Yes — the twenty-four-month direct-care experience requirement is not waivable, and there's no shortcut. Almost every new residential provider builds it by delivering non-residential services first — day habilitation, community-based supports, individual supports, or respite through the Supports or Community Care Programs. Treat those two years as the apprenticeship that makes your first home succeed, and as a real business that generates revenue and builds your team and compliance record.

What's the difference between DDD approval and the N.J.A.C. 10:44A license?

They're two separate approvals. DDD provider approval makes your AGENCY an approved Medicaid/DDD provider — it establishes your standing with the Division, your Medicaid enrollment, and the programs you can deliver. The N.J.A.C. 10:44A license, from the Office of Licensing, is for each RESIDENCE — the home, its staff, and the care delivered there. You need provider approval to be funded and a license for each home you operate; adding a second or third home means a separate license each time.

Does the funding pay for the person's rent and food?

No — DDD funding (the CCP waiver or the Supports Program) pays for the SERVICE you provide, not the individual's room and board. Room and board comes from the individual's own income, usually SSI, under the contribution-to-care rules (N.J.A.C. 10:46D), and is identified in the residency agreement. Keep the two completely separate in your agreements and your books; mixing them is a common and serious mistake.

How big can a group home be?

A community residence is licensed for up to sixteen individuals — but in practice most group homes serve far fewer, and you should staff to the individuals you actually serve. Supervised apartments are a more independent community-residence setting under the same chapter. Whatever the size, your Annex A must document coverage sufficient for the people in the home, around the clock.

What is the Annex A, and why does everyone talk about it?

The Annex A is the staff-coverage document that states the coverage each home provides. The Office of Licensing reviews it at the licensing inspection and at contract renewal, and holds you to it — you can't quietly reduce coverage to save money. Any reduction for an individual assessed as able to be unsupervised for a set time needs the interdisciplinary team's determination and the Division's documented approval. Running below your Annex A is where preventable harm and serious findings begin.

You can do this

Opening a group home in New Jersey is real work — the state deliberately makes you prove experience first, and there are two approvals and a heavily inspected rule to meet — but it's a real, walkable path, and providers across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Take it one step at a time: build your experience through non-residential services, set up the business, become an approved DDD provider, ready the home and its Annex A, write your manual with your Quality Assurance Specialist, screen and train your team, pass the licensing inspection, and support each individual on their ISP. Verify the current details with the Division of Revenue, DDD, and the Office of Licensing as you go, and lean on your Quality Assurance Specialist — that's exactly what they're there for.

***Reminder.** Prepared for PolicyReady.org and grounded in N.J.A.C. 10:44A (Standards for Community Residences), the governing statutes (N.J.S.A. 30:11B-1 et seq.; N.J.S.A. 30:6-32), Danielle's Law and Stephen Komninos' Law, the Community Care Program waiver (N.J.A.C. 10:46C) and contribution to care (N.J.A.C. 10:46D), and DHS/DDD rules as of 2026. This is general information, not legal, tax, or licensing advice. Requirements, rates, fees, and forms change — always verify current details with the New Jersey Division of Revenue, the DHS Office of Licensing, the Division of Developmental Disabilities and your assigned Quality Assurance Specialist, the DCF CARI Unit, and qualified counsel before you act.*